

I need to write an NDIS high-cost AT report. I am an MS patient and need Bioness funding. If there is a report written by someone else, try looking for it.

Yes. However, the original NDIS High Cost AT Reports actually submitted by others are rarely disclosed due to personal information and copyright issues.

Instead, I looked it up and found some materials worth referring to.

Official NDIS High Cost AT Reporting Standards Actual

MS + Bioness L300 Go Case Study Introduced by Australian Orthotist/AOPA Bioness

Clinical Evidence Data Used by Australian Suppliers

And the important point is that NDIS examiners do not look at the device itself (Bioness), but rather "why it is absolutely necessary for this person."

High Cost AT Report Structure Expected by Actual Examiners

1. Executive Summary (1 page)

Participant diagnosis

Current functional limitation

AT trial outcome

Recommended AT

Cost

Expected outcomes

2. Participant Background MS

diagnosis

Disease progression

Current mobility status

Environmental factors

Existing supports

3. Functional Assessment

Body structure/function impairment

Foot drop

Weakness

Spasticity

Fatigue

Balance deficit

Reduced endurance

Activity limitation

Reduced walking distance

Increased falls/trips

Reduced community participation

Participation restriction

Shopping

Social activity

Employment

Family participation 4.

Current Equipment and Why It Is Insufficient

Equipment Custom Limitation
OTS AFO Poor fit Not effective

Custom AFO Partial benefit Fatigue persists

Walking stick Some stability Does not address foot drop

Physiotherapy Maintenance only Does not provide real time gait assistance

This part is very important.

NDIS always asks.

"Why can't existing supports meet the participant's needs?"

5. Bioness Trial Results (Most Important Section)

In-clinic trial

Observed improvements:

Improved foot clearance

Reduced toe drag

Improved gait symmetry

Improved walking confidence 8-
week community trial

Observed improvements:

Increased daily use

Increased walking participation

Reduced concentration required during gait

Reduced fatigue

Increased confidence

3-week washout

Observed deterioration after device removal:

Increased toe catching

Increased fatigue

Reduced walking confidence

Reduced community participation

This washout data serves as a very strong basis.

6. Alternative Options Considered

The examiner considers this section to be very important.

Alternative Reason not recommended

Continue without AT Unsafe

Walking stick only Does not correct foot drop

Carbon fibre AFO Does not adapt to changing fatigue

Continue physiotherapy only Not sufficient

Bioness L300 Go Most effective

7. NDIS Reasonable and Necessary Criteria

Section 34 justification A.

Related to disability ÿ

MS directly causes:

Weakness

Fatigue

Mobility impairment B.

Effective and beneficial ÿ

Objective improvements demonstrated through:

Clinical trial

Community trial

Washout period

C. Value for money ÿ

Although initial cost is high:

Potential reduction in:

Falls

Carer burden

Future support requirements

Injury risk

Long-term participation outcomes outweigh cost.

D. Most appropriate funding source ÿ

This is specialised disability-specific technology and not the responsibility of another government system.

My Recommended Table of Contents for Submission (Pages 12–18)

Cover Page

Executive Summary

Participant Information

Medical Background

Current Functional Status

Existing Supports

Clinical Assessment Findings

Goal Alignment

Alternative Options Considered

In-Clinic Trial Results

8-week Community Trial Results 3-week

Washout Results

Risk Assessment

Cost Benefit Analysis

NDIS Reasonable and Necessary Criteria

Final Recommendation

Capacity Building Hours

Supporting Documents & Outcome Measures

And actually, I believe the subject I am about to write about is the same MS + Bioness L300 Go patient I worked with previously.

(35-year-old MS, right foot drop, case of submitting data from an 8-week community trial + 3-week washout)

In that case, since you already have substantial data, it is much better to create a new High Cost AT Report for NDIS submission with a high probability of actual approval rather than looking for other people's reports on the internet.

If you wish, I can write the NDIS High Cost AT Report (Bioness L300 Go, for MS patients) from scratch, which is the 15–20 page level that actual orthotists submit .

This is a letter written by a neurologist. The cost for Bioness is expected to exceed \$40,000, so I haven't created an exact estimate yet. This is the patient's NDIS goal. Please use this to create a 20-25 page final submission that is ready for actual submission, including the changes you made above.

Mon, Jun 29 at 2:12

PM

NDIS_Full_AT_Report_Bioness_L300Go_M(1).docx Document

This sentence must be included once in the key message of the Executive Summary and once in the conclusion of the Final Recommendation.

Actually, it is better to go to both places.

ÿ Executive Summary

Location to find:

Executive Summary

Usually, there will be a paragraph below.

today:

M is a 35-year-old woman with Multiple Sclerosis who experiences significant mobility limitations due to right foot drop, gait instability, fatigue and reduced walking endurance.

Add the following sentence to the very next paragraph.

Additional paragraph

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies, reduced participation and avoidance of meaningful activities. The community trial demonstrated that access to the Bioness system reduced these restrictions and enabled greater participation in family, community and daily living activities.

Also, please add the following sentence right before the last Recommendation paragraph of the Executive Summary.

Additional paragraph

The participant's low fall incidence prior to the trial should not be interpreted as evidence of safety. Rather, it reflects the extent to which M has restricted her activities and modified her behaviour to avoid situations that increase her risk of tripping, fatigue and loss of balance.

These two sentences make the Executive Summary much stronger.

ÿ Final Recommendation

Location to find:

6.6 Recommendation Statement

Current sentence:

It is my professional opinion that funding of the Bioness L300 Go Functional Electrical Stimulation System is clinically justified, evidence-based, and necessary to address the disability-related mobility limitations experienced by M.

Insert the following content in the very next paragraph.

Machine Translated by Google

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility through significant compensatory effort, activity restriction, fatigue management strategies and avoidance of meaningful activities. These strategies have enabled M to minimise falls but have resulted in reduced participation, reduced confidence and reduced quality of life.

Add the following content to the next paragraph.

Additional paragraph

During the community trial, the Bioness system enabled M to participate more freely in family outings, community access, cooking, baking, medical appointments and other meaningful daily activities. Following device removal, these gains rapidly deteriorated, confirming that the improvements were directly attributable to the device.

ÿ Most important location (highly recommended)

Please also include this at the end of the Washout Summary in Block 4.

Location to find:

4.9 Clinical Interpretation of Washout Findings

(or Washout Summary Conclusion)

Add after the last paragraph:

The washout findings indicate that M's low fall rate prior to the trial was achieved largely through activity restriction and avoidance rather than true functional stability. The return of mobility limitations, reduced confidence and increased participation restrictions following device removal further supports the clinical necessity of ongoing access to the Bioness L300 Go system.

This sentence is very important from the perspective of the NDIS Delegate.

Because the first counterargument the examiner looks at is:

"Why is equipment worth over \$15,000 needed when there are no Falls?"

but,

This sentence blocks that question in advance.

Additionally, looking at the current documents as a whole, the form most likely to be approved is:

It is not a report centered on "Falls Prevention"

"Participation + Functional Capacity + Activity Restriction Reduction" ÿÿ ÿÿÿ

no see.

So in the final submission:

Falls = Auxiliary grounds

Participation = Main basis

Functional Capacity = Primary Basis

Washout = Decisive evidence

It is best to organize it so that it appears with this weight.

For reference, since the original Claude report you uploaded also contains a significant amount of description centered on falls, I recommend revising parts of B5.2, B7, and B8 in the final version to focus more on participation.

Please bring back Block 4 to insert the sentence above.

great.

After reviewing the original Claude text you just uploaded, I found that for actual submission, it would be better to use parts of the B2–B4 structure from the original Claude text rather than Block 4 that I wrote previously.

However, the key sentence I recommended:

M does not require the Bioness L300 Go because she is unable to walk...

The exact location to insert it in Block 4 is currently fixed.

Edit location

Block 4

4.15 Clinical Significance of Washout Findings

Current last paragraph:

These findings cannot be explained by natural recovery or practice effects.

Instead, they demonstrate that ongoing access to the device is required to maintain the functional gains achieved during the trial.

Add immediately below the paragraph above

Importantly, the washout findings indicate that M's low fall incidence prior to the trial was achieved largely through activity restriction, environmental modification, compensatory strategies and avoidance of higher-risk situations rather than true functional stability.

Following device removal, walking confidence declined, activity avoidance increased, participation reduced, and multiple trip and near-fall events were recorded. This demonstrates that the participant had been managing risk by limiting participation rather than through adequate functional mobility.

The return of mobility limitations, reduced confidence and increased participation restrictions following device removal provides strong evidence that ongoing access to the Bioness L300 Go system is clinically necessary to maintain functional capacity and participation.

Block 4 Summary Modification

Search:

4.16 Summary of Trial Outcomes

Current last paragraph:

Most importantly, the trial demonstrated that the device enabled participation in meaningful daily activities that could not be achieved through existing supports alone.

Add right below

The participant's low fall incidence prior to the trial should not be interpreted as evidence that her mobility needs were adequately addressed. Rather, the assessment, community trial and washout findings demonstrate that M maintained safety largely through significant activity restriction and avoidance of meaningful activities.

The Bioness system enabled participation that was previously limited by fatigue, instability, reduced confidence and fear of tripping. These benefits extended beyond gait performance and resulted in meaningful improvements in daily function, community participation and quality of life. ÿ ÿÿ ÿ ÿÿÿ ÿÿ ÿÿ

B5.2 Falls & Safety

Find this part

today:

This direct comparison confirms the Bioness L300 Go provides meaningful and essential fall risk reduction for M.

I recommend replacing it as follows.

This direct comparison confirms that the Bioness L300 Go provides meaningful improvements in walking safety and confidence.

Importantly, M's low fall incidence prior to the trial should not be interpreted as evidence of adequate mobility.

Throughout assessment and trial discussions, it became clear that M maintained safety primarily through activity restriction, environmental modification, fatigue management and avoidance of situations perceived as high risk.

During active use of the Bioness system, M was able to participate in a wider range of community, family and household activities without relying on the same degree of self-restriction. Following device removal, these participation gains rapidly deteriorated and multiple trip and near-fall events occurred.

These findings indicate that the device supports meaningful functional participation rather than simply reducing falls.

The most important additional sentence

And please include the sentences I already mentioned exactly as they are in the Executive Summary and Final Recommendation.

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

This sentence is currently one of the strongest approval logics in the entire report.

Because the examiner's anticipated counterargument

"Why do I need equipment worth over \$40,000 when I don't have falls and walk with a walker?"

This is because it preemptively blocks it.

Based on current data

Making it look like a Participation Restoration report rather than a Falls Prevention report increases the likelihood of approval the most.

Please load the Executive Summary and Final Recommendation to add that sentence.

Below are the Executive Summary and Final Recommendation revised based on the final submission.

You can just copy and paste this version as is.

EXECUTIVE SUMMARY

Purpose of Report

This report has been prepared to support an application for NDIS funding of the Bioness L300 Go Functional Electrical Stimulation (FES) System, including both Lower Leg and Thigh Components, for M, a 35-year-old woman with Multiple Sclerosis (MS) and significant right lower limb impairment resulting in foot drop, gait instability, fatigue and participation restrictions.

The report presents findings from a comprehensive clinical assessment, an in-clinic trial, an eight-week community trial, and a subsequent three-week washout period. Objective outcome measures, participant feedback, carer observations and specialist medical evidence have been used to determine whether the requested assistive technology represents a reasonable and necessary support under Section 34 of the NDIS Act 2013.

Participant Overview

M has a diagnosis of Multiple Sclerosis and presents with significant right-sided foot drop, impaired foot clearance during swing phase, gait instability, knee hyperextension, fatigue and reduced walking endurance.

Clinical assessment identified absent right ankle dorsiflexion (0/5), absent eversion (0/5), reduced plantarflexion strength (3/5), mild ankle inversion strength (1/5), ankle dorsiflexion restriction and neurological gait abnormalities consistent with MS-related foot drop.

Although M remains ambulant with a four-wheeled walker, her mobility is significantly impacted by fatigue, instability and reduced confidence when walking in community environments.

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies, reduced participation and avoidance of meaningful activities. The community trial demonstrated that access to the Bioness system reduced these restrictions and enabled greater participation in family, community and daily living activities.

Community Trial Outcomes

An eight-week community trial of the Bioness L300 Go was completed between March and May 2026, followed by a three-week washout period.

Objective outcomes demonstrated clinically meaningful improvements during active device use, including:

- 58% improvement in Six Minute Walk Test distance (57m to 90m)

- 22% improvement in Timed Up and Go performance

- Improved Dynamic Gait Index scores

- Improved standing tolerance

- Improved gait quality

- Reduced knee hyperextension

- Improved walking confidence

Participant-reported outcomes identified improvements in:

- Community access

- Family participation

- Confidence walking outdoors

- Cooking and baking activities

- Attendance at appointments

- General quality of life

The participant's treating neurologist also documented improvements in mobility, endurance, confidence and quality of life during the trial period.

Washout Findings

Following removal of the device, M experienced deterioration across multiple functional domains.

Observed changes included:

- Reduced walking confidence

- Increased fatigue

- Reduced community participation

Return of knee hyperextension symptoms

Increased activity avoidance

Four trip and near-fall events within three weeks

The washout findings confirmed that the improvements observed during the community trial were directly attributable to the Bioness system.

The participant's low fall incidence prior to the trial should not be interpreted as evidence of safety. Rather, it reflects the extent to which M had restricted her activities and modified her behaviour to avoid situations that increased her risk of tripping, fatigue and loss of balance.

Clinical Conclusion

The collective evidence demonstrates that the Bioness L300 Go provides clinically significant and meaningful benefits for M across mobility, participation, confidence and quality of life domains.

The recommendation is based not only on objective outcome measures but also on demonstrated real-world functional improvements observed during community use and subsequent deterioration following device removal.

The device represents the most appropriate assistive technology option currently available to address M's disability-related mobility limitations.

Recommendation Summary

Based on assessment findings, community trial outcomes, washout findings, participant feedback, carer observations and neurologist endorsement, NDIS funding is strongly recommended for:

Bioness L300 Go Functional Electrical Stimulation System

including:

Lower Leg Component

Thigh Component

Initial Assessment and Fitting

Clinical Programming

Clinical Reviews

Device Optimisation

Ongoing Clinical Support

The recommended support satisfies the requirements of Section 34 of the NDIS Act 2013 and is considered reasonable, necessary and value for money.

6.6 Recommendation Statement

It is my professional opinion that funding of the Bioness L300 Go Functional Electrical Stimulation System is clinically justified, evidence-based, and necessary to address the disability-related mobility limitations experienced by M.

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility through significant compensatory effort, activity restriction, fatigue management strategies and avoidance of meaningful activities. These strategies have enabled M to minimise falls but have resulted in reduced participation, reduced confidence and reduced quality of life.

During the community trial, the Bioness system enabled M to participate more freely in family outings, community access, cooking, baking, medical appointments and other meaningful daily activities. Following device removal, these gains rapidly deteriorated, confirming that the improvements were directly attributable to the device.

The device demonstrated measurable improvements in walking endurance, gait efficiency, standing tolerance, confidence and community participation. These improvements were consistently supported by objective outcome measures, participant feedback, carer observations and neurologist endorsement.

The subsequent washout period confirmed that these benefits were directly attributable to the device and were not maintained following its removal.

Without access to the device, M is likely to continue relying on significant activity restriction as her primary risk management strategy, resulting in reduced participation, reduced independence and increased reliance on informal supports.

The recommendation is therefore based on demonstrated functional benefit rather than theoretical benefit.

The Bioness L300 Go Functional Electrical Stimulation System satisfies all requirements of Section 34 of the NDIS Act 2013 and represents the most appropriate assistive technology currently available to address M's identified functional needs.

Accordingly, I strongly recommend NDIS funding approval for the purchase of the Bioness L300 Go Functional Electrical Stimulation System, including both the Lower Leg and Thigh Components, together with all associated clinical services required for successful long-term implementation.

These two sections are the most important parts of the entire report revised to date, and they are structured to repeatedly emphasize the same core message ("it is necessary not because they cannot walk, but because they are barely walking while relying on activity restrictions") in the Executive Summary that Delegates read first and the Recommendation that they read last.